



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : عيبر احمد حسن محمد
المحافظة : القاهرة الحي / المدينة : مصر القديمه
رقم البطاقة : 27808070103744 رقم التليفون : 01115487552
العمر (تاريخ الميلاد) : 07-08-1978 المهنة : ربه منزل
العنوان : القاهرة
تاريخ تسجيل الإستمارة : 23-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : جراحات أورام تدخل جراحي لاستئصال كامل /تحفظى للثدى +تفريغ قسم العلاج :
الغدد الليمفاوية و/ او الغدد الحارسة [13155]
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: غير احمد حسن محمد
National ID: 27808070103744



تقرير طبي

السيد

تحتية طبية وبعد,,

بشأن تقرير طبي للمريض

إمضاء للطبيب المعتمد من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
---:--- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Mastectomy or breast-conserving surgery with axillary lymph node
dissection and/or sentinel lymph node biopsy

الوقت :

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاثنين

SEX: ♀ AGE: 47

LV Dimensions	AO, CUSP	1.5
LV Wall Thickness	1.5	
LV Diastolic Function	DD grade II	
SWMA	SWMA	
LA and Ao Root Dimensions	1.5	
Mitral Valve	1.5	
Aortic Valve	1.5	
Tricuspid Valve	1.5	
Pulmonary Valve	1.5	
Right Side and Systolic Function	1.5	
Intracardiac Masses and Effusion	1.5	

DR.

Chaitin

Name : Abeer Ahmed Hassan Mohammed
Branch : Dokki

Date : 04-08-2026
Medical No. : 431668

Musculoskeletal:

Normal FDG activity is seen over the axial skeleton. No focal active blastic or lytic bone lesions detected

Conclusion:

Positive study for FDG avid Lt breast neoplastic disease with no FDG avid metastatic deposits.

Thank you for referral

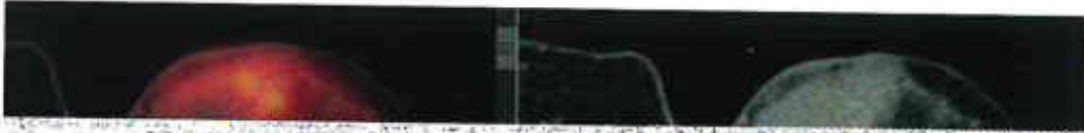
Doaa Ibrahim
MD of Nuclear Medicine

Walid Omar, MD
Prof. Nuclear Medicine

Call Center **19656**

Name : Abeer Ahmed Hassan Mohammed
Branch : Dokki

Date : 04-08-2026
Medical No. : 431668



Head and Neck:

- * Normal diffuse brain activity with no hypermetabolic brain lesions.
- * Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx, showing normal appearance.
- * Physiologic FDG uptake is seen in submandibular and parotid salivary glands, showing normal appearance.
- * Otherwise, Normal size, enhancement & symmetrical appearance of thyroid glands with no FDG avid lesions detected.
- * Normal appearance of the neck muscles & vessels.
- * No hypermetabolic cervical lymphadenopathy detected.
- * No FDG avid neck soft tissue mass lesions.

Chest:

- * No hypermetabolic mediastinal lymphadenopathy.
- * No FDG avid lung nodules or masses detected.
- * No metabolically active pericardial or pleural lesions detected.
- * Physiologic FDG uptake is noted in the heart, showing normal shape.

Abdomen and Pelvis:

- * No FDG avid hepatic focal lesions detected.
- * No FDG avid splenic focal lesions seen.
- * Physiologic FDG excretion is seen in the kidneys and bladder.
- * Normal appearance of the pancreas, aorta, I.V.C and both supra-renals with no FDG avid lesions detected.
- * No hypermetabolic abdominal or pelvic lymphadenopathy.
- * No FDG avid ascites or abdominal collections noted.
- * Rt adnexal cyst about 3x2.6cm

Call Center **19656**

Name : Abeer Ahmed Hassan Mohammed
Branch : Dokki

Date : 04-08-2026
Medical No. : 431668

Whole body PET/CT EXAMINATION:

Clinical History:

- 48 years old patient diagnosed with Lt breast cancer
- PET/CT requested for assessment.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 2.5..

Report:

Reference liver uptake 3.5



Breast and axilla:

- * FDG avid Lt breast UIQ lesion measuring about 8x5.7cm, SUVmax 3.6, the mass infiltrates overlying skin but no definite infiltration of underlying pectoralis muscle
- * Lt axillary LNs average 1.6cm, with preserved hilum, SUVmax 1
- * No definite FDG avid lesions at Rt breast
- * Rt axillary LNs of average 1cm with preserved hilum, no appreciable FDG uptake

Patient Name	عبيد احمد حسن	Case No.	345481
Sex/Age	Female 48 Y	Patient No.	25421
Registered Date	02/8/2026 10:06:45 AM	Referred By Dr.	Interventional Radiology
Validation Date	17/8/2026 01:14:08 PM	Branch	Gustave Roussy
Reporting Date	26/8/2026 01:34:05 PM	Pathology No.	2429/26

Pathology Report

Clinical Data: Left breast UIQ mass (BIRADs 5) with left axillary lymph node.

Nature of specimen:

- 1- Left breast mass, Tru-cut biopsy, collected on 02/08/2026, at 1:50 pm, cold ischemia time: 20 seconds.
- 2- Left axillary lymph node, Tru-cut biopsy, collected on 02/08/2026, at 1:52 pm, cold ischemia time: 20 seconds.

Gross Examination

Two specimens were received as follow:

- 1- Fragmented grey-white tissue cores, ranging in size between 0.2 cm & 0.8 cm long. Totally submitted.
- 2- One grey-white tissue core measured 1 cm long. Totally submitted.

Microscopic Examination

- 1- Histopathological examination reveals fragmented cores of tissue showing wide necrosis & hemorrhage, involved by high grade malignant epithelial deposits.
- 2- Histopathological examination reveals core of lymphoid tissue showing reactive changes & hemorrhage. Single cluster of atypical, degenerated cells is notable.

Diagnosis

1- Left breast mass, Tru-cut biopsy:

- Fragmented cores of wide necrosis & hemorrhage involved by high grade malignant epithelial deposits.

2- Left axillary lymph node, Tru-cut biopsy:



عقبة رقم (4) - الدور الأول - مبنى رقم (987) - عواريش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misor El Qadima - Cairo, Egypt

Patient Name	عبير احمد حسن	Case No.	345481
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Reporting Date	26/8/2026 01:34:05 PM	Pathology No.	2429/26

- Lymphoid tissue with reactive changes, hemorrhage & single cluster of atypical degenerated cells.

**** Immunophenotyping for (GATA-3, CK7, CK20, etc.....) is mandatory to confirm its origin.**

**** Tightly correlate with clinical & radiological findings.**

Signature

A. Prof. Dr. Rana El-Saeed.



غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورليس النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Coraiche - Mize El Qadima - Cairo, Egypt

Patient Name: Abeir Ahmed Hasan Mohamed	Exam Date: 7/21/2026
Patient ID: 30213839	Accession: MA260000033
Date Of Birth: 8/7/1978	Procedure: MA Mammography Diag + US Bilat
Gender: F	
Age: 47Y 11M	

Bilateral full field digital mammography with complementary high resolution breast ultrasound :

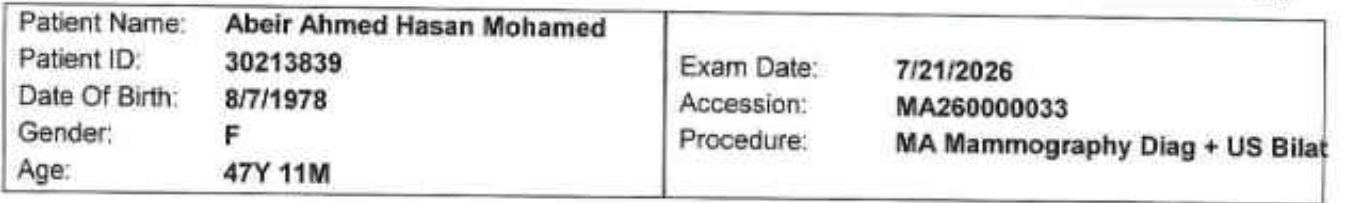
There are scattered areas of fibroglandular densities in both breasts.ACR C/B.
 Mild diffuse parenchymal mottling on US assessment,the inner central portion of the left breast is seen implicated by a large well defined hyperdense mass Iclosely adherent to pectoralis mass shadow .
 The mass assumes a partially microlobulated partially indistinct margin .
 On mammogram the mass measures about 10:0 x6.1 cm in diameters & is located about 9.0 X5.0cm distance from the nipple.
 No pathological microcalcifications elicited in either breast.
 No masses seen in right breast .
 Complementary US revealed:

A large solid defined irregular shaped hypoechoic mass located at 12/11 oclock of the left breast
 The mass measures about 8.2 cm in diameter.
 The mass casts strong posterior enhancement denoting its high cellularity.
 The mass is higly vascular on color doppler assessment.
 Focal edematous changes of the surrounding breast parenchyma evident by increased attenuation of the subcutaneous fatty tissues.
 Pathological left axillary lymph nodes showing focal cortical thickening(level 1) .
 No solid masses or cysts seen in right breast .
 No ductectasia
 Normal skin thickness & contour of both breasts as well as both nipple areolar complexes.
 Right likely non specific axillary lymph nodes.

Impression:

Left breast malignant looking mass as described with suspected pathological right axillary lymph nodes.**BIRADS 5** for histopathological correlation.

MRI breast may be warranted to exclude or confirm pectoralis muscle invasion.
 Mild bilateral mammary adenosis& left breast benign calcifications.BIRADS 2.



١ شارع سكة الإمام - السيدة زينب - القاهرة
ص. ب : ١ الدواوين - مصر
الهاتف : 19057 (202)
فاكس : 25 35 1745 (202)
57357Egypt



جمهورية مصر العربية
بطاقة تعريف شخصية



أحمد حسن محمد
١٢ ش محمود طه
دار السلام - القاهرة



٢٧٨٠٨٠٧٠١٠٣٧٤٤

J17393254



٢٠٢٢/٠٤

٢٧٨٠٨٧٧٠١٠٣٧٤٤

حاصلة على دبلوم تجارة



مزاوجة

مستلمة

أنثى

ناصر احمد محمد حشيش

البطاقة مارية حتى ٢٠٢٢/٠٤/٢٢



جوستاف
روسي
في قسم الجراحة

جراحة
9-٥٥



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية
مصر

09/22/2026

D

MDT FORM

Name:	عبد الله عبد الله	MRN:	12885
Age:	48 Y	ID:	27808070103744
Menopausal Sta	POST	Phone:	01001614407
NRS SCORE:	1	Marital Status:	MARRIED
BMI:	30	Family History:	+VE
Gender		Reason Of MDT:	
CO Morbidity:	DM	NEW CASE	
Surgical History:	* NAD	ECOG Performance Status	0 @ 1 0 2 0 3 0 4

Clinical Data:

BX: METAPLASTIC C SARCOMATOID TYPE
ER: 0 PR: 0 HER2: 0 KI67:

History:

Sonamamography:

DYNAMIC MRI ON BOTH BREASTS
LT UIQ MASS 8X6 CM WITH SURROUNDING EDEMA
ENLARGED 2 LNS

Biomarker/Molecular

Meta Workup:

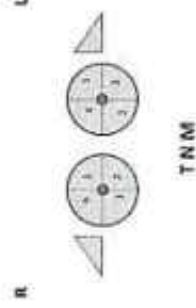
CT CAP FREE

Recommendations:

LT WLE WITH MATRIX ROTATION + SLNB AS LARGE BREAST SIZE

MDT Outcome & Plan

LT WLE WITH MATRIX ROTATION + SLNB AS LARGE BREAST SIZE



Right Side

Left Side

Q1

Q2

Q3

Q4

Q1

Q2

Q3

Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. طارق عبد الحليم	د. طارق عبد الحليم		



09/22/2026

MDT FORM

Name:	Fatma Abdelaziz Ali Sobh	MRN:	6015
Age:	62 Y	ID:	26409260100967
Menopausal Sta	POSTM	Phone:*	01013016277
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:		POSTNEOADJ	
Surgical History:*	NONE	ECOG Performance Status	
		0 0@ 1 0 2 0 3 0 4	

Clinical Data:

BX:

ER: a/8 PR: a/8 HER2: -ve KI67: 25

History:

postm rt breast cancer ct3 n1 mo ldc2 erf/8 pr/8 her2-ve ki 25 clips
inserted cesm rt ulq mass m 50x60mm
PT REC 4 AC 12 WEEKS TAXOL LAST 5/9/2026

Sonomammography:

Biomarker/Molecular

Met Workup:

Recommendations:

FOR RT CBS+SLNB AFTER STEREOTACTIC WIRE LOCALIZATION

MDT Outcome & Plan

FOR RT CBS+SLNB AFTER STEREOTACTIC WIRE LOCALIZATION

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. محمد عبد الحادي	د. محمد عبد الحادي		



S C A N

OPINION :

Left UIQ pathologically proven malignant looking lesion as described above with ipsilateral axillary lymphadenopathy . **BIRADS 6**
Mildly enlarged right axillary lymph nodes .
Right scattered foci of punctuate parenchymal enhancement categorized as **BIRADS 2** based on the morphological and kinetic data.
Bilateral mammary fibroadenosis .

Dr. ENGY ADEL,MD

Prof. of Radiology, Cairo University

Defining BIRADS classification of breast lesions according to the American College of Radiology:

<u>Category</u>	<u>Suspected Cancer Risk</u>
BIRADS 0 : Non-informative study.	
BIRADS 1 : Normal study.	0 %
BIRADS 2 : Benign findings.	0 %
BIRADS 3 : Probably benign.	2 %
BIRADS 4 : Probably malignant.	50 %
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	



S C A N

Name: Abeer Ahmed Hassan

ID: 5325302

Referred by: Prof. Mohamed Al Nabarawy

High Field Dynamic MRI of Both Breasts

Technique:

- Axial T2 (plate 1). Axial STIR (plate 2).
- Sagittal postcontrast 3D TFE of the right breast (plate 3).
- Sagittal postcontrast 3D TFE of the left breast (plate 4).
- Dynamic multiphase post-contrast study done in 8 minutes with MIP reconstruction (plate 5,6). Time intensity curves (plate 7).

Findings:

- Scattered mixed densities of glandular condensation.
- Bilateral moderate background glandular enhancement.
- Left UIQ large well-defined lesion of intermediate SI in T2 and heterogeneous bright SI in STIR measures about 8x6cm with surrounding edematous changes and mid distortion ,in contrast study showed irregular ring like enhancement ,the kinetic data showed mixed types of curves moderate initial rise of the curve followed by plateau at 120 seconds (Type 2 curve) and continuous steady rise with no wash out (Type 1 curves).
- Right scattered foci of punctuate parenchymal enhancement ,the kinetic data showed continuous steady rise with no wash out (Type 1 curves).
- Normal skin thickness and contour.
- No nipple or skin enhancement.
- Enlarged left axillary lymph nodes 2cm showed cortical thickening
- There are likely non-specific right axillary lymph nodes with some of the right side are enlarged .

Patient Name	عبر احمد حسن	Case No.	384323
Sex/Age	Female 48 Y	Patient No.	25421
Registered Date	13/9/2026 12:52:18 PM	Referred By Dr.	Surgery
Validation Date	16/9/2026 10:03:55 AM	Branch	Gustave Roussy
Reporting Date	17/9/2026 01:34:42 PM	Pathology No.	2429/26

Biomarker report

Clinical Data: Left breast UIQ mass (BIRADs 5) with left axillary lymph node.

Nature of specimen:

- 1- Left breast mass, Tru-cut biopsy, collected on 02/08/2026, at 1:50 pm, cold ischemia time: 20 seconds.
- 2- Left axillary lymph node, Tru-cut biopsy, collected on 02/08/2026, at 1:52 pm, cold ischemia time: 20 seconds.

Gross Examination

Two specimens were received as follow:

- 1- Fragmented grey-white tissue cores, ranging in size between 0.2 cm & 0.8 cm long. Totally submitted.
- 2- One grey-white tissue core measured 1 cm long. Totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for: CK7 clone (SP52), CK20 clone (SP33), GATA3 clone (L50-823), SOX-10 clone (SP267), CD34 clone (Qbend10), P63 clone (4A4) ER clone (1E2), PR clone (SP1), Her2 clone (4B5).

Microscopic Examination

- 1- Histopathological examination reveals fragmented cores of tissue showing wide necrosis & hemorrhage, involved by high grade malignant epithelial deposits.
 - Histologic type: Metaplastic carcinoma. (Sarcomatoid type).
 - Grade: 3 (Differentiation 3, Pleomorphism 3, Mitosis 3).
 - Necrosis: Present.
 - 2- Histopathological examination reveals core of lymphoid tissue showing reactive changes & hemorrhage. Single cluster of atypical, degenerated cells is notable.
- ** IPT (Controls show appropriate reactivity):**
- CK7: Expressed only in glandular structures.

Patient Name	عبد الحميد حسن	Case No.	384323
Sex/Age	Female 48 Y	Patient No.	25421
Registered Date	13/9/2026 12:52:18 PM	Referred By Dr.	Surgery
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- CK20: Negative.
- GATA-3: Weakly positive in tumor cells.
- SOX-10: Negative.
- CD34: Negative in tumor cells.
- P63: Focally weakly positive in tumor cells.
- CK: Positive in tumor cells.
- ER: Negative tumor cells (score 0/8).
- PR: Negative tumor cells (score 0/8).
- Her2: Negative membrane reaction in tumor cells (score 0).

Diagnosis

1- Left breast mass, Tru-cut biopsy:

- Findings are consistent with metaplastic carcinoma (Sarcomatoid type).

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).

2- Left axillary lymph node, Tru-cut biopsy:

- Lymphoid tissue with reactive changes, hemorrhage & single cluster of atypical, degenerated cells.

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

** Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive



Gustave Roussy International-Egypt Pathology Lab
جورستاف رويسى - مصر - مصر - مصر - مصر - مصر - مصر - مصر - مصر - مصر - مصر



Patient Name	عبد الله حسن	Case No.	384323
Sex/Age	Female 48 Y	Patient No.	25421
Registered Date	13/9/2026 12:52:18 PM	Referred By Dr.	Surgery
Validation Date	16/9/2026 10:03:55 AM	Branch	Gustave Roussy
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tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr. Maha Guinei, MD, PhD.

Patient Name	عبد الحادي حسن	Case No.	360789
Sex/Age	Female 48 Y	Patient No.	25421
Registered Date	26/8/2026 03:41:14 PM	Referred By Dr.	Interventional Radiology
Validation Date	02/9/2026 03:52:58 PM	Branch	Gustave Roussy
Reporting Date	05/9/2026 10:29:27 AM	Pathology No.	2429/26

Pathology Report

Clinical Data: Left breast UIQ mass (BIRADs 5) with left axillary lymph node.

Nature of specimen:

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- 2- Left axillary lymph node, Tru-cut biopsy, collected on 02/08/2026, at 1:52 pm, cold ischemia time: 20 seconds.

Gross Examination

Two specimens were received as follow:

- 1- Fragmented grey-white tissue cores, ranging in size between 0.2 cm & 0.8 cm long. Totally submitted.
- 2- One grey-white tissue core measured 1 cm long. Totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for; CK7 clone (SP52), CK20 clone (SP33) and GATA3 clone (L50-823).

Microscopic Examination

- 1- Histopathological examination reveals fragmented cores of tissue showing wide necrosis & hemorrhage, involved by high grade malignant epithelial deposits.
- 2- Histopathological examination reveals core of lymphoid tissue showing reactive changes & hemorrhage. Single cluster of atypical, degenerated cells is notable.

** IPT (Controls show appropriate reactivity):

- CK7: Expressed only in glandular structures.
- CK20: Negative.
- GATA-3: Negative.

Diagnosis



Patient Name	عبر احمد حسن	Case No.	360789
Sex/Age	Female 48 Y	Patient No.	25421
Registered Date	26/8/2026 03:41:14 PM	Referred By Dr.	Interventional Radiology
Validation Date	02/9/2026 03:52:58 PM	Branch	Gustave Roussy
Reporting Date	05/9/2026 10:29:27 AM	Pathology No.	2429/26

1- Left breast mass, Tru-cut biopsy:

- Fragmented cores of wide necrosis & hemorrhage involved by high grade malignant neoplasm (Negative for CK20 & GATA-3) with expression of CK7 in glandular elements.

2- Left axillary lymph node, Tru-cut biopsy:

- Lymphoid tissue with reactive changes, hemorrhage & single cluster of atypical, degenerated cells.

** Case for further immunophenotyping (P63, CD34, SOX-10, ER, etc....).

** Tightly correlate with clinical & radiological findings.

Signature

Prof. Dr. Maha Guimei.



جوستاف روسي روتسي انترناتيونال - مصر - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Alor El-Qadima - Cairo, Egypt

ECHO REPORT

NAME: عیسیٰ احمد خان

DATE: 3 / 8 / 2026

ID NUMBER: /

SEX: ♀ AGE: 47

EF	70%	55-75%	PWT	1.0	0.6-1.1cm
LVEDD	4.9	3.5-5.8cm	AO.R	2.9	3-3.7cm
LVESD		2-4cm	LA	3.8	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	1.1	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions ☒ normal

LV Wall Thickness ☒ normal

LV Diastolic Function DD grade II

SWMA ☒ normal

LA and Ao Root Dimensions ☒ normal

Mitral Valve ☒ normal

Aortic Valve ☒ normal

Tricuspid Valve ☒ normal

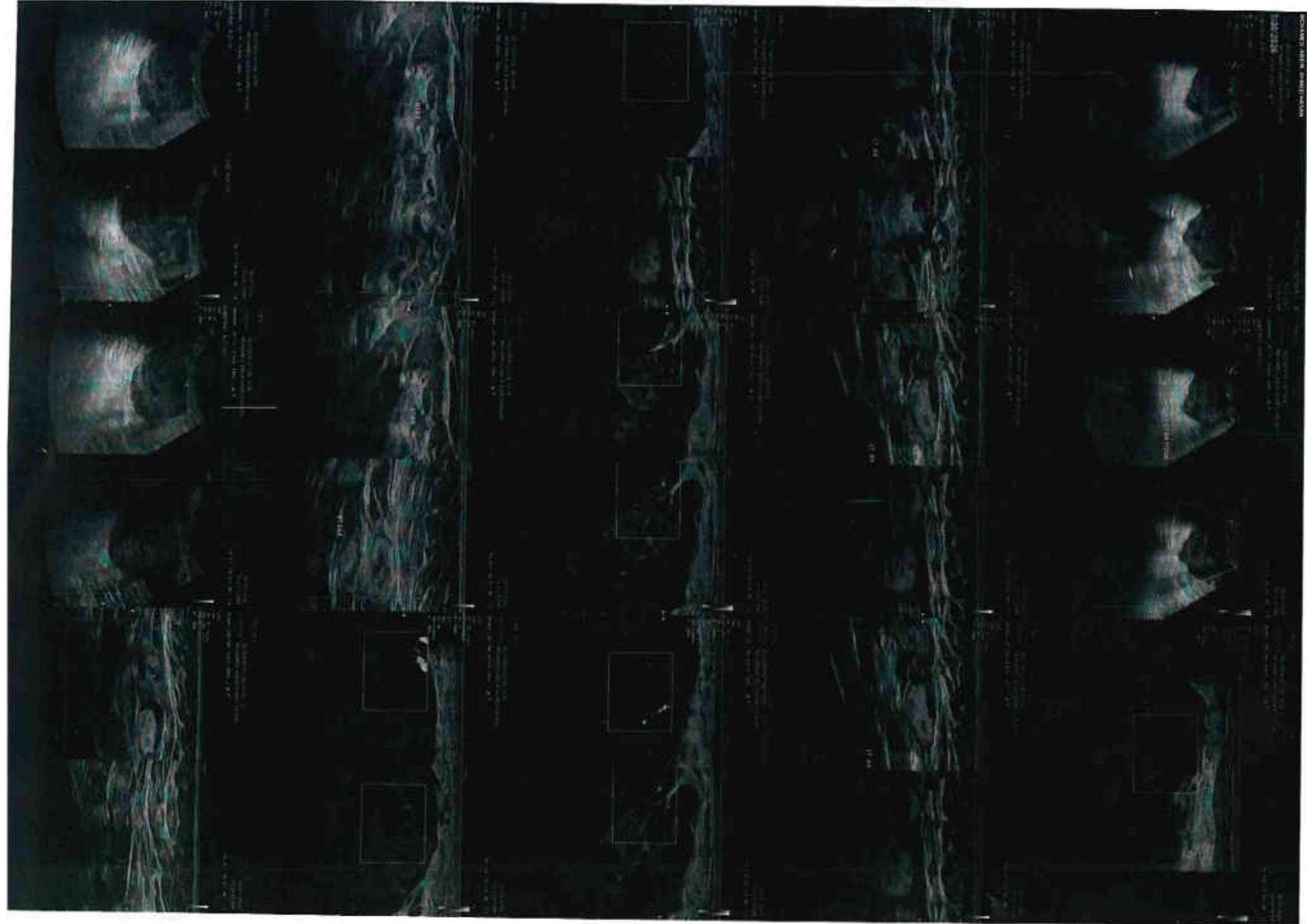
Pulmonary Valve ☒ normal

Right Side and Systolic Function ☒ normal

Intracardiac Masses and Effusion ☒ normal

DR:

جستاف



Name : Abeer Ahmed Hassan Mohammed
Branch : Dokki

Date : 04-08-2026
Medical No. : 431668

Musculoskeletal:

Normal FDG activity is seen over the axial skeleton. No focal active blastic or lytic bone lesions detected

Conclusion:

Positive study for FDG avid Lt breast neoplastic disease with no FDG avid metastatic deposits.

Thank you for referral

Doaa Ibrahim
MD of Nuclear Medicine

Walid Omar, MD
Prof. Nuclear Medicine

Name : Abeer Ahmed Hassan Mohammed
Branch : Dokki

Date : 04-08-2026
Medical No. : 431668



Head and Neck:

- * Normal diffuse brain activity with no hypermetabolic brain lesions.
- * Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx, showing normal appearance.
- * Physiologic FDG uptake is seen in submandibular and parotid salivary glands, showing normal appearance.
- * Otherwise, Normal size, enhancement & symmetrical appearance of thyroid glands with no FDG avid lesions detected.
- * Normal appearance of the neck muscles & vessels.
- * No hypermetabolic cervical lymphadenopathy detected.
- * No FDG avid neck soft tissue mass lesions.

Chest:

- * No hypermetabolic mediastinal lymphadenopathy.
- * No FDG avid lung nodules or masses detected.
- * No metabolically active pericardial or pleural lesions detected.
- * Physiologic FDG uptake is noted in the heart, showing normal shape.

Abdomen and Pelvis:

- * No FDG avid hepatic focal lesions detected.
- * No FDG avid splenic focal lesions seen.
- * Physiologic FDG excretion is seen in the kidneys and bladder.
- * Normal appearance of the pancreas, aorta, I.V.C and both supra-renals with no FDG avid lesions detected.
- * No hypermetabolic abdominal or pelvic lymphadenopathy.
- * No FDG avid ascites or abdominal collections noted.
- * Rt adnexal cyst about 3x2.6cm

Name : Abeer Ahmed Hassan Mohammed
Branch : Dokki

Date : 04-08-2026
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Whole body PET/CT EXAMINATION:

Clinical History:

- 48 years old patient diagnosed with Lt breast cancer
- **PET/CT requested for assessment.**

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROIs and the normal threshold is < 2.5 .

Report:

Reference liver uptake 3.5



Breast and axilla:

- * FDG avid Lt breast UIQ lesion measuring about 8x5.7cm, SUVmax 3.6, the mass infiltrates overlying skin but no definite infiltration of underlying pectoralis muscle
- * Lt axillary LNs average 1.6cm, with preserved hilum, SUVmax 1
- * No definite FDG avid lesions at Rt breast
- * Rt axillary LNs of average 1cm with preserved hilum, no appreciable FDG uptake

Patient Name	عبر احمد حسن	Case No.	345481
Sex/Age	Female 48 Y	Patient No.	25421
Registered Date	02/8/2026 10:06:45 AM	Referred By Dr.	Interventional Radiology
Validation Date	17/8/2026 01:14:08 PM	Branch	Gustave Roussy
Reporting Date	26/8/2026 01:34:05 PM	Pathology No.	2429/26

Pathology Report

Clinical Data: Left breast UIQ mass (BIRADs 5) with left axillary lymph node.

Nature of specimen:

- 1- Left breast mass, Tru-cut biopsy, collected on 02/08/2026, at 1:50 pm, cold ischemia time: 20 seconds.
- 2- Left axillary lymph node, Tru-cut biopsy, collected on 02/08/2026, at 1:52 pm, cold ischemia time: 20 seconds.

Gross Examination

Two specimens were received as follow:

- 1- Fragmented grey-white tissue cores, ranging in size between 0.2 cm & 0.8 cm long. Totally submitted.
- 2- One grey-white tissue core measured 1 cm long. Totally submitted.

Microscopic Examination

- 1- Histopathological examination reveals fragmented cores of tissue showing wide necrosis & hemorrhage, involved by high grade malignant epithelial deposits.
- 2- Histopathological examination reveals core of lymphoid tissue showing reactive changes & hemorrhage. Single cluster of atypical, degenerated cells is notable.

Diagnosis

1- Left breast mass, Tru-cut biopsy:

- Fragmented cores of wide necrosis & hemorrhage involved by high grade malignant epithelial deposits.

2- Left axillary lymph node, Tru-cut biopsy:



شركة راسم (4) - الطور ١٩٧٧ - مبنى رقم ٩٨٧٦ - كورنيش النيل - مصر القاهرة - القاهرة - مصر
Room No. (4) - First Floor, Building No. 9876, Nile Corniche - Misor El Qahira - Cairo, Egypt

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- Lymphoid tissue with reactive changes, hemorrhage & single cluster of atypical degenerated cells.

**** Immunophenotyping for (GATA-3, CK7, CK20, etc.....) is mandatory to confirm its origin.**

**** Tightly correlate with clinical & radiological findings.**

Signature

A. Prof. Dr. Rana El-Saeed.



خريطة رقم (43) - القاهرة - مبنى رقم (987) - مركز ليش النيل - مصر القديمة - القاهرة - مصر
 Room No. (43) - First Floor-Building No. 987, Nile Corniche - Alvir El Qalena - Cairo, Egypt

Patient Name:	Abeir Ahmed Hasan Mohamed	Exam Date:	7/21/2026
Patient ID:	30213839	Accession:	MA260000033
Date Of Birth:	8/7/1978	Procedure:	MA Mammography Diag + US Bilat
Gender:	F		
Age:	47Y 11M		

Bilateral full field digital mammography with complementary high resolution breast ultrasound :

There are scattered areas of fibroglandular densities in both breasts.ACR C/B.
 Mild diffuse parenchymal mottling on US assessment.the inner central portion of the left breast is seen implicated by a large well defined hyperdense mass lclously adherent to pectoralis mass shadow .
 The mass assumes a partially microlobulated partially indistinct margin .
 On mammogram the mass measures about 10.0 x6.1 cm in diameters & is located about 9.0 X5.0cm distance from the nipple.
 No pathological microcalcifications elicited in either breast.
 No masses seen in right breast .
 Complementary US revealed:

A large solid defined irregular shaped hypoechoic mass located at 12/11 oclock of the left breast
 The mass measures about 8.2 cm in diameter.
 The mass casts strong posterior enhancement denoting its high cellularity.
 The mass is highly vascular on color doppler assessment.
 Focal edematous changes of the surrounding breast parenchyma evident by increased attenuation of the subcutaneous fatty tissues.
 Pathological left axillary lymph nodes showing focal cortical thickening(level 1) .
 No solid masses or cysts seen in right breast .
 No ductectasia
 Normal skin thickness & contour of both breasts as well as both nipple areolar complexes.
 Right likely non specific axillary lymph nodes.

Impression:

Left breast malignant looking mass as described with suspected pathological right axillary lymph nodes.**BIRADS 5** for histopathological correlation.
 MRI breast may be warranted to exclude or confirm pectoralis muscle invasion.
 Mild bilateral mammary adenosis& left breast benign calcifications.BIRADS 2.



مركز سرطان الأطفال
Feinbockgasse - Egypt

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Children's Cancer Hospital
Feinbockgasse - Egypt

Patient Name:	Abeir Ahmed Hasan Mohamed	Exam Date:	7/21/2026
Patient ID:	30213839	Accession:	MA2600000033
Date Of Birth:	8/7/1978	Procedure:	MA Mammography Diag + US Bilat
Gender:	F		
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Final Report by: Maha Hussein

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حاصلة على دبلوم تجارة



مزاوجة

مستلمة

أنثى

ناصر احمد محمد حشيش

البطاقة مارية حتى ٢٠٢٢/٠٤/٢٢

